MIST- MEDICAL INSTITUTE FOR SCREENING TEST

PAEDIATRICS SUBJECT WISE TEST DATED ON 7 MAY 2026

QUESTIONS

Q1. A 2-year old child is brought for developmental evaluation. The pediatrician evaluates the child's milestones. Absence of which of these milestones should increase suspicion of a possible developmental delay?
a) Standing with support
b) Complete bowel control
c) Riding a tricycle
d) Walking on stairs maturely

Q2. When evaluating a child's midarm circumference using a 3-color Shakir's tape, which of these values would indicate possibility that the child has moderate malnutrition?
a) 105 mm
b) 110 mm
c) 112 mm
d) 124 mm

Q3. During developmental evaluation of 1-month old infant, the pediatrician evaluates the head circumference and chest circumference of the child and informs the mother that the head circumference is more than the chest circumference. The mother is curious to know when the two will become equal. This is most likely to be achieved at the age of
a) 3 months
b) 6 months
c) 9 months
d) 12 months

Q4. A term newborn is resuscitated after birth and his birth weight is 3 kg on day 1 of life. In the first few days, this infant may physiologically lose what percentage of his birth weight?
a) 1-2%
b) b. 2-4%
c) 1-4%
d) 5-10%

Q5. In a child whose height on the growth chart is less than the 3rd percentile, all investigations to evaluate the underlying pathological cause for his short height fail to reveal any abnormality. His skeletal maturation shows his bone age to be less than his chronological age. Which of the following statements is likely to be false for this cause of short stature?
a) Most common cause of short stature
b) Parents height likely to be short
c) Likely to have a normal final height
d) Puberty may be affected

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Q6. A 6-year old boy with short stature has short parents. His parents are curious to know his approximate final height. His father's height is 160 cm and mother's height is 152 cm. His mid-parental height will likely be

a) 160.5 cm
b) 162.5 cm
c) 167.5 cm
d) 172.0 cm

Q7. Most children speak in jargons at what age?

a) 6 months
b) 9 months
c) 15 months
d) 24 months

Q8. A child has diarrhea, irritability, alopecia, and sharply-demarcated, dry, scaly red-color plaques in periorificial location, sparing the upper lip. These features in the child are likely to result from deficiency of

a) Copper
b) Zinc
c) Iron
d) Selenium

Q9. A newborn with intractable seizures has been given phenobarbitone and phenytoin, although with no response. Administration of which of the following vitamins in the newborn may abort these otherwise refractory seizures?

a) Riboflavin
b) Niacin
c) Pyridoxine
d) Thiamine

Q10. Most children are able to attain a height double than their birth length by the age of

a) 6-12 months
b) 12-18 months
c) 18-24 months
d) 36-48 months

Q11. A malnourished child with bilateral pedal edema and weight for height below -3z scores is being assessed by a pediatrician. Which of the following third criteria will he use to confirm a diagnosis of severe acute malnutrition?

a) Weight for age
b) Height for age
c) Chest circumference
d) Midarm circumference

Q12. A severely malnourished child with hypothermia, oliguria and poor skin turgor is assessed and diagnosed to be in shock. Which of the following will be the fluid of choice for administration in this child?

a) Ringer Lactate
b) Normal Saline
c) ReSoMal
d) Ringer Lactate in 5% Dx

Q13. Which of these is used as the best diagnostic criteria for severe wasting in children?
a) Weight for age below 2SD
b) Weight for height below 3 SD
c) Height for age below 3 SD
d) Weight for age below 3 SD

Q14. Which of these is used as the best diagnostic criteria for stunting in children?
a) Weight for age below 2SD
b) Weight for height below 3 SD
c) Height for age below 2 SD
d) Height for age below 3 SD

Q15. A mother who is not able to directly breast feed her child is advised to feed her child using breast milk expressed in a clean container. The mother enquires for how long can she keep breast milk viable in a clean container at room temperature. The pediatrician advises that she can keep milk for
a) 6-8 hours
b) 3-4 hours
c) 16-24 hours
d) 24-48 hours

Q16. Which of these is not considered normal in a 12-hour old newborn and should raise concern of an underlying disorder?
a) Heart rate of 150/minute
b) No meconium passed
c) Faint jaundice in the eyes
d) Respiratory rate of 50/minute

Q17. All of the following can be considered contraindications of breastfeeding except
a) HIV positive mother on ART
b) Baby with classic galactosemia
c) Baby with phenylketonuria
d) Mother of the baby taking illicit drugs

Q18. A woman visits a pediatrician’s clinic worried of the presence of smooth, blue- purpulish patches present on the back, more concentrated on the lower back of the baby, since birth. What will be the pediatrician’s advice?

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a) Reassurance
b) Surgical opinion
c) Complete blood count and CRP testing
d) Start indomethacin for a week

Q19. A 4-day old newborn develops blotchy red spots on the skin of the trunk with overlying firm, yellow-white bumps. Baby is otherwise active and taking feeds well. The lesions are most likely to consist of which of these cells?

a) Basophils
b) Mast cells
c) Macrophages
d) Eosinophils

Q20. Which of these vitamin deficiencies is expected in an exclusively breastfed baby?
a) Vitamins A and C
b) Vitamins D and K
c) Vitamins C and K
d) Vitamins D and E

Q21. A 2.2 kg baby is hypothermic and being attended by the pediatrician in charge. He advises the kangaroo mother care technique. This evidence-based method is often considered the standard of care for low-birth-weight or premature infants to keep them warm and includes all of the following except
a) Early discharge and support
b) Prolonged skin-to-skin contact

c) Massaging the periumbilical area
d) Early exclusive breastfeeding

Q22. A newborn delivered to a mother with poorly controlled type 2 diabetes is diagnosed as LGA (large for gestational age). His weight is likely to be
a) More than 10the percentile
b) More than 50th percentile
c) More than 75th percentile
d) More than 90the percentile

Q23. During the grand rounds, a pediatrician is discussing neonatal reflexes. He is performing these reflexes in an 8-month old infant. Which of these statements indicates a concern?
a) Rooting reflex absent
b) Moro reflex present
c) Parachute reflex present
d) Asymmetric tonic neck reflex absent

Q24. A 38-week infant is delivered by section and develops mild breathing difficulty with some nasal flaring shortly after birth. His APGAR is 5 and 6 at 1 and 5 minutes after birth. Chest X ray shows perihilar streakiness, prominent interlobal fissures and minimal pleural effusion. This disorder is most likely to be due to
a) Meconium aspiration
b) Surfactant deficiency
c) Congenital infection
d) Persistence of fetal lung fluid

Q25. A baby after birth undergoes examination. The pediatrician evaluates APGAR score at 1 minute after birth. All of the following criteria are used in APGAR scoring of a newborn after birth except
a) Cry
b) Color
c) Tone
d) Heart rate

Q26. A newborn with tachypnea and subcostal retraction is diagnosed with respiratory distress syndrome. Which of these should be considered the most significant risk factor for this disorder?
a) Female gender
b) Pregnancy complication
c) Dyselectrolytemia
d) Prematurity

Q27. A 29-weeks pregnant woman is likely to deliver in the next 1 week. The following corticosteroid can be administered to reduce risk of severe RDS in the baby after birth

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a) Mometasone
b) Dexamethasone
c) Prednisolone
d) Methylprednisolone

Q28. A 31-week preterm baby while in NICU develops abdominal distension, feeding intolerance, and bloody stools. X ray abdomen shows gas in the bowel wall and some evidence of portal venous gas as well. Which of these is unlikely to be a finding in the blood reports of this newborn?
a) Hyponatremia and thrombocytopenia
b) Thrombocytopenia and metabolic acidosis
c) Metabolic alkalosis and hypernatremia
d) Hyponatermia and metabolic acidosis

Q29. A preterm baby, 1850 gm at birth, is born by a complicated delivery and required resuscitation after birth. Cry was feeble, APGAR was < 3 at 5 minutes and baby required CPAP for 24 hours in the NICU. After birth, baby had 2 seizures and hypotonia. Which of the following neuropathological injury is most likely in the baby?
a) Sagittal infarcts
b) Multifocal necrosis
c) Periventricular leukomalacia
d) Basal ganglia infarcts

Q30. Which of the following is a correct statement for umbilical cord?
a) One artery and one vein
b) Two veins and one artery
c) Two veins and two arteries
d) Two arteries and one vein

Q31. A 30-week preterm baby was shifted to the NICU on day 1 after the baby had 2 episodes of seizures, bulging fontanelle, and cardiorespiratory instability in the resuscitation room. The cranial ultrasound showed intracranial bleed. Most intracranial bleeds in these newborns are likely to originate from
a) Basal ganglia
b) Cerebellum
c) Brainstem
d) Germinal matrix

Q32. A 1.2 kg preterm baby is admitted in the nursery. The baby is to be started on IV fluids. The volume of fluid in the first 24 hours would be
a) 95 ml
b) 110 ml
c) 125 ml
d) 135 ml

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Q33. A newborn delivered at 31 weeks is stable on day 3 and the pediatrician plans to start trophic feeds in the baby. Which of these is the best mode of administering milk to the baby?
a) Breast feeds
b) Palidai feeds
c) Tube feeds
d) Spoon feeds

Q34. A preterm newborn, 1.7 kg weight, develops jaundice on the face and eyes within 24 hours of birth. The most likely cause of this neonatal jaundice will be
a) Congenital hypothyroidism
b) Rh incompatability hemolysis
c) Physiological jaundice
d) Neonatal hepatitis

Q35. Neonatal cholestasis is an important cause of prolonged neonatal jaundice. Prolonged jaundice which satisfies the criteria of “neonatal cholestasis” is conjugated bilirubin persisting in a newborn for more than
a) 3 days after birth
b) 1 week after birth
c) 2 weeks after birth
d) 4 weeks after birth

Q36. The technique shown in the picture is commonly used for reducing serum bilirubin in neonatal jaundice. Light used in this technique is most likely effective if it has a wavelength of

a) 400-420 nm
b) 460-490 nm
c) 510-540 nm
d) 540-560 nm

Q37. A baby is born to a diabetic mother. The infant is large size, macrosomic newborn. The baby cried after birth and is shifted to nursery for evaluation and observation. What should be the time to perform glucose screening in this otherwise asymptomatic baby?

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a) Avoid as baby asymptomatic
b) Should be performed after 24 hours
c) Should be performed after 72 hours
d) Should be performed early, usually within 1 hours of birth

Q38. During neonatal resuscitation, which of these is the correct criteria for starting chest compression?
a) HR < 100/minute
b) HR < 60/minute
c) RR > 60/minute
d) RR > 80/minute

Q39. A preterm newborn after delivery is handed to a neonatologist. The baby cried at birth. During resuscitation of this baby, what will be the first priority of the neonatologist?
a) Give epinephrine
b) Intubate the baby
c) Provide the baby warmth
d) Establish IV access

Q40. A neonate presents with severe respiratory distress after birth. The neonate has a scaphoid abdomen and a mediastinal shift of cardiac impulse to the right. Which of these is the correct statement for the most likely disorder?
a) Failure of closure of pleuroperitoneal membrane
b) Commonly associated with oligohydramnios
c) Bag and mask ventilation should be treatment of choice
d) Commonly associated with tracheoesophageal fistula

Q41. Which of these is considered the most common cause of neonatal and early childhood mortality?
a) Congenital malformations
b) Diarrhea
c) Prematurity and its complications
d) Perinatal asphyxia

Q42. A preterm newborn delivered at 33 weeks to a mother with gestational diabetes develops breathing difficulty and grunting shortly after birth. The chest X-ray of the neonate revealed reticulogranular pattern and air bronchograms. The mother did not receive antenatal steroids. The baby has a respiratory rate of 68/minute, good respiratory efforts, though chest retractions are present. The Sa02 was 90% on room air. Which of the following is the most appropriate initial management step?
a) Intubation and mechanical ventilation
b) Nasal CPAP
c) Oxygen by a nasal cannula
d) IV dexamethasone

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Q43. A 2-day old newborn baby was lethargic, intermittent jittery, had a high pitch cry and had an episode of apnea. A bedside glucose test showed blood glucose 38 mg/dl. What is the best initial management for this infant?
a) Feeding milk and recheck blood sugar after 30 minutes
b) Milk 6 hourly
c) IV 10% Dx bolus followed by IV glucose infusion
d) Recheck sugar in 4 hours

Q44. Which of these congenital heart diseases can be complicated with Eisenmenger syndrome?
a) Tetrology of Fallot
b) Patent Ductus Arteriosus
c) Truncus Arteriosus
d) Coarctation of Aorta

Q45. A 7-month infant is brought to the emergency department with a history of cyanotic spells while crying. The child was diagnosed with TOF at 3 months of age. The child had central cyanosis and a harsh systolic ejection murmur at the left upper sternal border. Which of the following is not an appropriate management of this cyanotic spell?
a) IV digoxin
b) IV metoprolol
c) Put child in knee chest position
d) IV phenylephrine

Q46. A 4-day old newborn is brought to the emergency with cyanosis that does not improve by giving oxygen. The baby was delivered to a diabetic mother. The chest X-ray reveals a narrow mediastinum and a heart shape resembling an "egg on a string" appearance. Which of these management options may improve outcomes in this newborn?
a) IV indomethacin
b) IV digoxin
c) IV Prostaglandin E1
d) High pressure oxygen

Q47. A child with cyanosis, fast breathing and poor weight gain is being evaluated. Chest X ray of the child is shown. Which is the most likely heart disease?

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a) TOF
b) Ebstein anomaly
c) TGA
d) TAPVR

Q48. Which of these congenital heart diseases is most likely associated with cyanosis, pulmonary oligemia on chest X ray and RVH on ECG is a 3-year child?
a) TOF
b) TAPVR
c) Tricuspid atresia
d) Truncus arteriosus

Q49. A child has bilateral cataracts and chorioretinitis, microcephaly, sensineural hearing loss and a congenital heart disease. Antenatal history of the mother of the child was significant for acute maculopapular rash, fever, and tender lymph node enlargement behind the ear at 7^{th} week of gestation. Which of these congenital heart diseases is most likely in the child?
a) TOF
b) Ebstein anomaly
c) PDA
d) TAPVR

Q50. An infant is brought to the emergency with cyanosis, feeding difficulties, excessive sweating during feeds, poor weight gain, and tachypnea. The infant is diagnosed with heart failure while echocardiogram revealed an underlying congenital heart disease as the cause of these symptoms. Which of these congenital heart diseases will be the least likely cause of these symptoms?
a) TOF
b) TGA
c) TAPVR
d) Tricuspid atresia

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Q51. A child is diagnosed as a case of triology of fallot on echocardiogram. Which of these cardiac abnormalities will not be seen in this disorder?
a) ASD
b) VSD
c) Pulmonary stenosis
d) RVH

Q52. A child is brought for evaluation in a pediatric OPD with pain in the legs while walking. His upper limb pulses were normal while lower limb pulses were feeble. Which of the statements below is not compatible with the most likely congenital heart disease?
a) An acyanotic congenital heart disease
b) Presentation of symptoms often in late childhood
c) Enlargement of collaterals often bypass aortic obstruction
d) Narrowing most common in ascending aorta

Q53. Which of these congenital heart diseases is most likely in a child with Turner syndrome?
a) VSD
b) Atrioventricular septal defect
c) Valvular pulmonary stenosis
d) Coarctation of aorta

Q54. A 4-month-old infant has rapid breathing and difficulty taking feeds shortly after birth. Examination findings include tachycardia, tachypnea and a prominent "machinery" murmur heard throughout the cardiac cycle. Echocardiogram reveals a large PDA with significant left to right shunting and left atrial enlargement. If left untreated, this child with a large PDA will be at the highest risk of which of these complications in late childhood?
a) Hypertrophic cardiomyopathy
b) Rupture of the ductus arteriosus
c) Eisenmenger syndrome
d) Cardiomegaly

Q55. Which of the following shunts in fetal circulation helps blood bypass the liver (portal circulation)?
a) Ductus Venosus
b) Ductus Arteriosus
c) Foramen Ovale
d) Umbilical artery

Q56. A 2.2 kg newborn was delivered at 38 weeks' gestation. The child had a weak cry at birth and had a heart rate of 70 bpm, irregular breathing with gasps and intermittent grunting. The tone is flaccid tone and there is no response to stimulus. What is the most appropriate next step in management?
a) Repeat tactile stimulation
b) Start PPV using ambu bag and mask
c) Intubate the baby

d) Chest compression with PPV

Q57. Identify the congenital heart disease seen in a child with typical chest X ray shown in the image :

a) Coarctation of aorta
b) TOF
c) TGA
d) Supracardiac TAPVR

Q58. A 14-month-old child is brought to the pediatric OPD with history of persistent diarrhea. Examination showed bilateral pedal edema, a MUAC of 105 mm, however the child passes the appetite test by consuming the provided Ready-to-Use Therapeutic Food (RUTF). The child appears alert and active. What is your diagnosis?
a) Moderate malnutrition
b) Chronic malnutrition
c) Uncomplicated Severe Acute Malnutrition
d) Complicated Severe Acute Malnutrition

Q59. A pediatrician is conducting a workshop for mothers explaining them the differences between growth and development. Which of the statements given below is the best illustration of "development" compared to "growth"?
a) Head circumference of infants increases 2 cm/month in first 3 months
b) Stunting is typically assessed by height for age
c) Children typically start climbing stairs from 2 years onwards
d) Breast feeding is the best form of nutrition in early infancy

Q60. Most children develop right and left handedness by the age of :
a) 6 months
b) 1 year
c) 2 years
d) 3 years

Q61. Which of these skull fontanelles is the last to close in a child?
a) Sphenoid fontanelle
b) Anterior fontanelle

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c) Posterior fontanelle
d) Mastoid fontanelle

Q62. A newborn is delivered at 37 weeks gestation to a mother with inconsistent prenatal care and a history of carbamazepine use during pregnancy for seizures. Examination of the child revealed a fleshy, sac-like protrusion on the lower lumbosacral region containing both meninges and neural tissue. Lower extremity tone was reduced. Most likely diagnosis is :
a) Craniorachischisis
b) Encephalocele
c) Meningocele
d) Myelomeningocele

Q63. Myelomeningocele in children are commonly co-associated with :
a) Sacral fistula
b) Ventricle hemorrhage
c) Hydrocephalus
d) Anencephaly

Q64. A woman with previous pregnancy complicated with a fetus having meningocele is planning her next pregnancy. She visits an antenatal clinic and seeks preventive measures for its recurrence. According to guidelines, what is the most appropriate recommendation for her?
a) Folic acid 0.4 mg daily
b) Folic acid 1 mg daily
c) Folic acid 4 mg daily
d) Folic acid 40 mg daily

Q65. A 6-month-old child with history of hydrocephalus underwent a ventriculoperitoneal (VP) shunt placement at birth. About a week later he was brought to the emergency with high fever, persistent vomiting, and reduced activity. The child was drowsy on examination. There was redness and swelling along the shunt tract. The pediatrician explains to the mother that these features are likely complications of shunt placement and could have been avoided with which of these procedures for hydrocephalus?
a) Lateral ventricular drainage
b) Aqueduct resection and reimplantation
c) Endoscopic third ventriculostomy
d) Fourth ventricular reimplantation

Q66. A neonate is brought to the emergency with high fever, poor feeding, extreme irritability and high-pitched cry. The CSF examination showed significantly elevated white cell counts, elevated proteins, and low glucose. A diagnosis of neonatal meningitis is made. Which of these would be least likely implicated cause?
a) S aureus
b) H. influenza
c) E . coli

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d) S. agalactIae

Q67. A 3.5 year-old child is brought to the emergency department after an episode of generalized tonic-clonic seizure that occurred at home. The child was febrile to touch and had evidence of otitis media. Neurological examination was normal, with a normal mental state, no nuchal rigidity or any evidence of post-ictal drowsiness. The child has no history of previous seizures or neurodevelopmental delays. Which of the following is the most appropriate next step in management?
a) Give long-term phenobarbitone for future seizure recurrence
b) Admit the child, lumbar puncture for CSF analysis
c) Reassurance and awareness on seizure with fever
d) Intermittent prophylaxis with clobazam next time with fever

Q68. Which of these is the drug of choice for the treatment of a child with recurrent episodes of sudden unresponsive staring spells, eyelid fluttering episodes and immediate return to normal activity after the episodes?
a) Phenobarbitone
b) Phenytoin
c) Sodium Valproate
d) Diazepam

Q69. A child is brought to the emergency with several episodes of early morning vomiting without nausea and episodes of ataxia since the past 3-4 weeks. The child has a broad-based gait, right-sided dysmetria on finger-to-nose testing, and bilateral papilledema. The MRI head revealed a contrast-enhancing mass in the midline of the cerebellum causing obstructive hydrocephalus. Which of the following is the most likely diagnosis?
a) Craniopharyngioma
b) Choroid plexus tumor
c) Medulloblastoma
d) Diffuse astrocytoma

Q70. All of the following are predominantly supratentorial brain tumors in children except:
a) Choroid plexus tumor
b) Medulloblastoma
c) Diffuse astrocytoma
d) Craniopharyngioma

Q71. While evaluating a newborn, a large right-side abdominal lump is noted. Which of these is the most likely cause?
a) Polycystic kidney
b) Nephroblastoma
c) Neuroblastoma
d) Multicystic dysplastic kidney

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Q72. A 12-month old infant with high-grade fever and irritability is diagnosed with UTI (pyelonephritis). She has a history of two previous episodes of documented UTIs which were treated with oral antibiotics. Examination revealed mild suprapubic tenderness. An MCU revealed reflux of urine into a dilated ureter with blunting of the calyceal fornices but without significant parenchymal thinning. What is the most appropriate next step in management for this patient?

a) Ureteral reimplantation surgery
b) Endoscopic subureteral bulking agents given through injections
c) Low-dose antibiotic prophylaxis
d) Wait and watch with serial MCU

Q73. A 2-year-old boy is brought by a worried mother to the pediatrician's office after she noticed a firm mass on the right side of his abdomen while bathing him. The child is otherwise asymptomatic. The mass was not tender, ballotable and did not cross the midline. Urinalysis shows microscopic hematuria while ultrasound abdomen showed a solid intrarenal mass. The most likely diagnosis is

a) Neuroblastoma
b) Nephroblastoma
c) Multicystic kidney
d) Pheochromocytoma

Q74. Which of these is the least likely site of metastatic spread of a neuroblastoma is a 3-year-old child?

a) Lungs
b) Liver
c) Bone
d) Lymph nodes

Q75. Henoch Schonlein purpura is a complex immune-mediated small vessel vasculitis involving blood vessels in various organ systems. Which of these is its most common manifestation in children?

a) Rash
b) Abdomen pain
c) Arthralgia
d) Renal involvement

Q76. Shown in the picture is a subtype of malnutrition. All of the following are correct about this malnutrition type except:

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a) Alert look
b) Poor appetite
c) Sugar baby appearance
d) Edema

Q77. Which of the following is the most commonly used treatment for a child with HSP (IgA vasculitis), rash, arthralgia and abdomen pain?
a) Azathioprine
b) Antibiotics
c) Corticosteroids
d) Monoclonal antibodies

Q78. A 2.5-year-old girl with periorbital edema has been passing "frothy" urine since the last 3 days. There is no history of fever, sore throat, or skin infections. Blood pressure is 105/60 mmHg. Laboratory results show 4+ proteinuria on dipstick and a serum albumin of 1.8 g/dL. What is your management?
a) Antibiotics
b) Prednisolone
c) Cyclophosphamide
d) IVIG

Q79. Which of these congenital obstructions of the urinary tract in children is associated with a “filling defect” in the bladder seen on MCU?
a) Ureterocele
b) PUV
c) U/L Congenital UPJ obstruction
d) B/L Congenital UPJ obstruction

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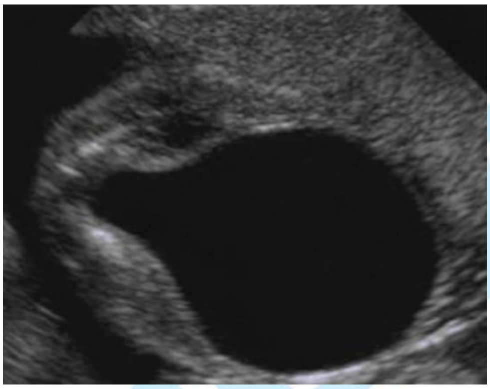
Q80. Ultrasound examination showing appearance of a "keyhole sign" in an infant is most likely indicative of



a) Hydronephrosis
b) PUV
c) Ureterocele
d) Ectopic ureter

Q81. A 6-year-old boy has been having recurrent episodes of pathological proteinuria and was prescribed 12-weeks course of steroid therapy. Despite treatment both proteinuria and edema persist. A renal biopsy is planned. Which of these observations is most likely in the histopathological examination of the biopsy specimen of this "steroid-resistant" state?
a) Effacement of podocyte foot processes
b) Normal histopathology
c) Mesangial expansion
d) Focal segmental scarring of some glomeruli (FSGS)

Q82. Which of these is considered the "gold standard" test for diagnosing vesicoureteric reflux (VUR) in children?
a) X ray abdomen
b) Ultrasound abdomen
c) VCUG
d) DTPA scan

Q83. A 7-year-old boy presents with episodes of gross hematuria which the mother has been noticing since the last 2 days. He also had a history of throat infection started 48 hours back. The child was hypertensive. Proteinuria was 2+. A renal biopsy was planned. Light microscopic examination revealed diffuse expansion of the extracellular matrix and hypercellularity in the mesangium. On electron microscopy prominent electron-dense deposits were seen within the mesangial and paramesangial regions. Immunofluorescence is likely to reveal predominant deposition of which of these in mesangium?
a) IgE
b) C4
c) IgA

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d) Mast cells

Q84. A 5-year-old boy is brought to pediatric OPD with history of cola-colored urine, periorbital puffiness and a blood pressure of 136/88 mmHg. He had developed pyoderma 3 weeks back and was prescribed topical fusidic acid application on skin infected. Laboratory tests show low C3 levels. Which statement regarding his condition is incorrect?
a) Corticosteroids can improve prognosis and is recommended
b) Commonly causes acute glomerulonephritis
c) C3 levels typically return to normal within 6–8 weeks
d) Antihypertensive treatment is advised

Q85. A 1-week-old neonate has abdominal distension and failed to pass meconium. There is a history of cystic fibrosis in his family. What is the most likely cause for the present neonatal signs/symptoms?
a) Anal stenosis
b) Rectal prolapse
c) Meconium ileus
d) Small left colon

Q86. Cystic fibrosis is most commonly reported due to a genetic defect which is located on the long arm of which chromosome?
a) Chromosome 3
b) Chromosome 7
c) Chromosome X
d) Chromosome 10

Q87. An 8-year-old child who is a known case of cystic fibrosis is brought to the pediatric OPD with progressively worsening productive cough associated with expectoration of green-colored sputum. He has a past history of recurrent chest infections. Which organism is the most common likely cause these chest symptoms?
a) Staphylococcus aureus
b) Pseudomonas aeruginosa
c) Burkholderia cepacian
d) Hemophilus influenzae

Q88. A 7-month-old infant is brought to the emergency department with repeated cough episodes and respiratory rate of 56 breaths per minute but no chest indrawing. He was taking feeds well and was active. What is the most appropriate classification based on this child's symptoms?
a) Cold and cough
b) Pneumonia
c) Severe Pneumonia
d) No Pneumonia

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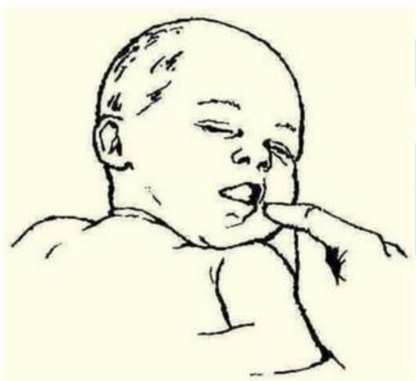
Q89. All of the following respiratory tract disorders may be cause of "wheezing" in a child except :
a) Croup
b) Pneumonia
c) Bronchiolitis
d) Asthma

Q90. A 1.5-year-old girl is brought to the pediatric clinic for evaluation of suspected pneumonia. At what respiratory rate (breaths per minute) would this child be classified as having tachypnea/pneumonia?
a) > 30
b) > 40
c) > 50
d) > 60

Q91. A mother brings her 1-year old child to a primary health center complaining that the child was having watery stools since last 2 days. On examination, the child was irritable, had a poor skin turgor, but was thirsty and eager to drink. According to IMNCI guidelines, which color category should this child be classified into?
a) Green
b) Yellow
c) Pink
d) Red

Q92. A baby was delivered after a cesarian section at 36 weeks of gestation. Which of these assessment scores is used to evaluate the baby at 1 minute and 5 minutes after birth?
a) Ballard score
b) Silverman Anderson Score
c) Downe score
d) Apgar score

Q93. Which neonatal reflex is being evaluated in the newborn?



a) Moro reflex
b) Rooting reflex

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c) Asymmetric tonic neck reflex
d) Symmetric tonic neck reflex

Q94. What is the recommended amount of Oral Rehydration Solution (ORS) to be given in a 3-year-old child with diarrhea and vomiting, and diagnosed with "some dehydration"?
a) 30 ml/kg over 1 hour
b) 75 ml/kg over 4 hours
c) 100 ml/kg over 7 hours
d) 20 ml/kg over 4 hours

Q95. In a newborn who has failed to pass meconium even after 48 hours of birth, which which disorder should be mainly suspected?
a) Duodenal atresia
b) Cholestasis
c) Hirschsprung's disease
d) Celiac disease

Q96. An 8-month-old infant with first wheezing is diagnosed with bronchiolitis. Which of the following is NOT typically a part of the management of this disorder?
a) Maintain hydration
b) Antibiotics
c) Humidified oxygen if saturations <92%
d) Supportive care

Q97. Which of the pharmacological options is considered the “gold standard” for long-term maintenance (controller) treatment for asthma in children?
a) Inhaled corticosteroids
b) Long-acting beta agonists
c) Short-acting beta agonists
d) Antibiotics

Q98. Squatting improves symptoms in which of these congenital heart diseases in children?
a) TOF
b) Tricuspid atresia
c) Hypoplastic left heart syndrome
d) None of these

Q99. Which of the statements mentioned below are not consistent with infantile polycystic kidney disease?
a) Autosomal dominant
b) Congenital liver often co-associated
c) PKHD1 gene mutation

d) Microcysts in cortex and medulla of the kidneys

Q100. Which of the following provoked seizures are most common in childhood?
a) Absence seizures
b) Juvenile myoclonic epilepsy
c) Generalized Tonic Clonic Seizures
d) Febrile seizures

ANSWERS

1	a	26	d	51	b	76	a
2	c	27	b	52	d	77	c
3	d	28	c	53	d	78	b
4	d	29	c	54	c	79	a
5	b	30	d	55	a	80	b
6	b	31	d	56	b	81	d
7	c	32	a	57	a	82	c
8	b	33	c	58	c	83	c
9	c	34	b	59	c	84	a
10	d	35	c	60	d	85	c
11	d	36	b	61	b	86	b
12	d	37	d	62	d	87	b
13	b	38	b	63	c	88	b
14	c	39	c	64	c	89	a
15	a	40	a	65	c	90	b
16	c	41	c	66	b	91	b
17	a	42	b	67	c	92	d
18	a	43	c	68	c	93	b
19	d	44	b	69	c	94	b
20	b	45	a	70	b	95	c
21	c	46	c	71	d	96	b
22	d	47	b	72	c	97	a
23	b	48	a	73	b	98	a
24	d	49	c	74	a	99	a
25	a	50	a	75	a	100	d